

Of course. Based on global and Indian-specific maternal health data, here is the prioritized list of fatal conditions during delivery, with a specific focus on their prevalence and impact in the Indian context.

### ### \*\*Prioritized List of Fatal Conditions in India\*\*

The order below reflects the most common and devastating causes of maternal mortality in India, based on public health data and reports.

#### #### \*\*1. Postpartum Haemorrhage (PPH)\*\*

**\*\*The #1 Killer in India\*\***

\* **\*\*Why it's the top priority:\*\*** PPH is the **\*\*leading cause of maternal death in India**, accounting for approximately 30-35% of all maternal mortality.\*\*

\* **\*\*Indian Context Factors:\*\***

\* **\*\*High Prevalence of Anemia:\*\*** A staggering **\*\*50-60% of Indian pregnant women\*\*** are anemic. Anemic women have less blood reserve and can tolerate very little blood loss before going into fatal shock.

\* **\*\*Delayed Access to Care:\*\*** Geographical and financial barriers can delay reaching a facility capable of managing PPH (e.g., performing surgery or providing blood transfusions).

\* **\*\*Substandard Care:\*\*** In some cases, inadequate management of the third stage of labour (active management is not universally practiced) contributes to preventable deaths from uterine atony.

#### #### \*\*2. Pre-Eclampsia & Eclampsia\*\*

**\*\*The "Silent" Hypertensive Crisis\*\***

\* **\*\*Why it's a major priority:\*\*** Hypertensive disorders are the **\*\*second leading cause**, responsible for about 20-25% of maternal deaths in India.\*\*

\* **\*\*Indian Context Factors:\*\***

\* **Lack of Routine Screening:** Inconsistent blood pressure monitoring during antenatal check-ups in rural or low-resource settings can lead to late diagnosis.

\* **Limited Access to Magnesium Sulfate:** This is the first-line drug for preventing and treating eclamptic seizures. Its availability in primary health centers is still a challenge.

\* **Cultural Factors:** Symptoms like swelling and headaches are often dismissed as normal parts of pregnancy, delaying seeking care.

### ### **3. Sepsis (Infections)**

#### **A Deadly Consequence of Unhygienic Conditions**

\* **Why it remains a critical threat:** Sepsis accounts for a significant **15-20%** of maternal deaths in India.

\* **Indian Context Factors:**

\* **Unhygienic Delivery Conditions:** This is a major risk factor, especially in home deliveries without a skilled birth attendant.

\* **High Rates of Unskilled Delivery:** Despite progress, a notable percentage of births are not attended by trained professionals who can maintain aseptic techniques.

\* **Unsafe Abortions:** Infections following unsafe termination of pregnancy are a significant contributor to sepsis-related deaths.

### ### **4. Obstructed Labour**

#### **A Preventable Tragedy**

\* **Why it's a priority:** While its absolute numbers have decreased, it remains a deadly issue, often linked to **cephalopelvic disproportion (CPD)** where the baby's head is too large for the mother's pelvis.

\* **Indian Context Factors:**

\* **Child Marriage and Early Pregnancy:** Though illegal, the practice persists in some regions. Young adolescents have underdeveloped pelvises, drastically increasing the risk of obstructed labour.

\* **Lack of Timely C-Section Access:** A delay in deciding to transfer to a facility or in performing a necessary emergency C-section can lead to uterine rupture, sepsis, and death for both mother and baby.

#### 5. Complications of Unsafe Abortion

##### A Largely Preventable Cause

\* **Why it's a specific concern:** Despite legalization, unsafe abortions are the 3rd leading cause of maternal mortality in India, contributing significantly to death from sepsis and hemorrhage.

##### Indian Context Factors:

\* **Stigma and Lack of Awareness:** Women, especially unmarried ones, may seek clandestine and unsafe procedures due to social stigma.

\* **Limited Access to Safe Services:** Knowledge about and access to legal, safe Medical Termination of Pregnancy (MTP) services is not uniform across the country.

#### 6. Amniotic Fluid Embolism (AFE) & Co-morbidities

##### The Unpredictable and The Underlying

\* **Amniotic Fluid Embolism (AFE):** While rare and unpredictable everywhere, its outcome in India is often worse due to the delay in accessing advanced ICU care and managing the consequent DIC (massive bleeding).

\* **Pre-existing Co-morbidities:** Conditions like heart disease are an emerging cause of maternal death in India as the average maternal age rises and the burden of non-communicable diseases increases.

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#### Summary Table for India

| Rank | Condition | Key Contributing Factors in India |

| :--- | :--- | :--- |

| **\*\*1\*\*** | **\*\*Postpartum Haemorrhage (PPH)\*\*** | **\*\*Widespread maternal anemia\*\***,  
delayed access to emergency care, lack of skilled birth attendants. |

| **\*\*2\*\*** | **\*\*Pre-Eclampsia & Eclampsia\*\*** | Lack of routine BP screening, delayed care-  
seeking, limited access to critical drugs like Magnesium Sulfate. |

| **\*\*3\*\*** | **\*\*Sepsis\*\*** | Unhygienic delivery practices, high rate of unskilled attendance,  
infections from unsafe abortions. |

| **\*\*4\*\*** | **\*\*Obstructed Labour\*\*** | **\*\*Early childbearing\*\***, malnutrition, delays in  
accessing emergency C-sections. |

| **\*\*5\*\*** | **\*\*Unsafe Abortion\*\*** | Social stigma, lack of awareness of legal rights, limited  
access to safe MTP services. |

**\*\*Conclusion for the Indian Context:\*\***

The priority list underscores that **\*\*most maternal deaths in India are preventable.\*\*** The solutions lie not only in advanced medical technology but in strengthening basic healthcare infrastructure: combating anemia, ensuring skilled birth attendance at every delivery, promoting institutional deliveries, and improving access to emergency obstetric care and safe abortion services. Public health initiatives targeting these specific areas have the highest potent